POSITION DESCRIPTION

1. Agency PD Number: PX3439 PCMC

2a. Reason for Submission
- [ ] Redescription
- [ ] New
- [ ] Update Supervisor Certification
- [ ] Reestablishment
- [ ] Other
- [x] Reorganization

3. Service
- [ ] Headquarters
- [x] Field

4. Employing Office Location: Phoenix, Arizona

5. Duty Station: PAIHS - PIMC

2b. Position Information (Complete all that apply)
- [ ] PCA
- [ ] Drug Testing
- [ ] Physical Exam Required

6. Fair Labor Standards Act
- [ ] Exempt
- [x] Nonexempt

7. Financial Statements Required
- [ ] OGE-278
- [ ] OGE-450
- [x] N/A

8. Subject to IA
- [x] Yes
- [ ] No

Acq Code: 00 | BUS Code: 1776 | Cyber Sec Code: 00 | FPL: 10

- [ ] Select Agent Access
- [ ] Incumbency Only
- [ ] Commissioned Corps

2c. Explanation (Show any positions replaced)

Realigned under Nursing Services. Replaces PX3439 PCMC, Nurse, GS-0610-10, dated 12/10/2019.

- [ ] Limited Term
- [ ] Limited Emergency

NTE Date:

9. Position Status
- [x] Competitive
- [ ] Excepted (*Specify in remarks*)
- [ ] SES (GEN)
- [ ] SES (CR)

10a. Administrative Code: GFLM262

10b. Job Requisition/Position Number

11. Position is:
- [ ] Supervisor/Manager (Code 2)
- [ ] Supervisor-CSRA (Code 4)
- [ ] Management Official (CSRA) (Code 5)
- [ ] Work Leader (Code 6)
- [ ] Team Leader (Code 7)
- [x] Neither (Code 8)

12. Position Sensitivity
- [ ] 1-Nonsensitive
- [ ] 2-Noncritical Sensitive
- [ ] 3-Critical Sensitive
- [ ] 4-Special Sensitive
- [x] 5-Public Trust/ Moderate
- [ ] 6-Public Trust/High Risk

13. Comp Level: NUG0

14. Job Code: PX3439

15. OPM Certification No.

16. Classified/Graded by:	Official Title of Position	Pay Plan	Occupational Series Code	Grade	Initials	Date
a. U.S. Office of Personnel Management						
b. Department, Agency or Establishment	Nurse	GS	0610	10	PM	02/28/2021
c. Recommended by Supervisor or Initiating Office	Nurse	GS	0610	10	CT	02/28/2021

17. Organizational Title of Position (*if different from official title*): Clinical Care Coordinator (PCMC)

18. Name of Employee (*if vacant, specify*)

19. Department of Health and Human Services

a. First subdivision: Indian Health Service, Phoenix Area Indian Health Service

b. Second subdivision: Phoenix Service Unit

c. Third subdivision: PHS Indian Medical Center, PHO/Phoenix Indian Medical Center (PIMC)

d. Fourth subdivision: Division of Nursing Services, Ambulatory Nursing Services

e. Fifth subdivision: PCMC Nursing

20. **Supervisory Certification**. *I certify that this is an accurate statement of the major duties and responsibilities of this position and its organizational relationships, and that the position is necessary to carry our Government functions for which I am responsible. This certification is made with the knowledge that this information is to be used for statutory purposes relating to appointment and payment of public funds, and that false or misleading statements may constitute violations of such statutes or their implementing regulations.*

a. Typed Name/Title/Employee ID of Immediate Supervisor

Carolyn Tapahe, RN
Supervisory Nurse, PCMC Nursing (Acting)

Signature: Carolyn R. Tapahe -S Digitally signed by Carolyn R. Tapahe -S Date: 2021.02.10 13:33:58 -07'00'

b. Typed Name/Title of Higher Level Supervisor or Manager (optional)

Candace Lee, RN, BSN, BC-CCTM
Assistant CNO, Ambulatory Nursing Services

Signature: Candace Lee -S Digitally signed by Candace Lee -S Date: 2021.02.10 14:59:42 -07'00'

21. **Classification/Job Grading Certification.** *I certify that this position has been classified/graded as required by the Title 5, U.S. Code, in conformance with standards published by the U.S. Office of Personnel Management or, if no published standards apply directly, consistently with the most applicable published standards.*

Typed Name and Title of Official Taking Action

Patricia Marquez, Classification Specialist
SWR Human Resources, PIMC

Signature: Patricia A. Marquez -S Digitally signed by Patricia A. Marquez -S Date: 2021.02.17 17:29:01 -07'00'

22. Position Classification Standards Used in Classifying/Grading Position

U.S. OPM Job Family Position Classification Standard for Professional Work in the Medical and Healthcare Group, 0600, Nursing 0610 series, September 2017; and General Schedule Leader Grade Evaluation Guide HRCD-5 June 1998 (cross-reference only).

Information for Employees: *The standards and information on their application are available in the human resources office. The classification of the position may be reviewed and corrected by the agency or the U.S. Office of Personnel Management. Information on classification/job grading appeals, and complaints on exemption from FLSA, is available from the human resources office of the U.S. Office of Personnel Management.*

23. Remarks: **Position is covered by P.L. 101-630 or P.L. 101-647 or both.** Incumbent must be suitable for Federal employment, as determined by fingerprint check results and appropriate background investigation. An incumbent found not suitable after hire will be removed from this position. **UFMS:** [ ] Yes [x] No

Conditions of employment – this position requires, at a minimum, Level 5 – MBI moderate risk level background investigation since the contents of the PD meets the Public Trust position criteria.

CSC: 000-000-000

24. *Description of Major Duties and Responsibilities and Evaluation Statement (See Attached)*

POSITION TITLE: Nurse
ORGANIZATIONAL TITLE: Clinical Care Coordinator (PCMC)
PP/SERIES/GRADE: GS-0610-10
ORG LOCATION: DHHS, IHS, Phoenix Area, Phoenix Service Unit, Phoenix Indian Medical Center, Division of Nursing Services, Ambulatory Nursing Services, PCMC Nursing
FLSA STATUS: Non-exempt (Per MOA)
JOB CODE: PX3439

INTRODUCTION

This position is located at the Phoenix Service Unit, Phoenix Indian Medical Center (PIMC), Phoenix, Arizona, and can be assigned to any location throughout the medical center/Phoenix Service Unit, i.e. Nursing Services, Ambulatory Services, Outpatient Departments, and patient care teams associated with the Medical Home. The incumbent is a member of an ambulatory medical home care team with both medical provider staff and other health care staff (clinical and non-clinical). Incumbent ensures patients at the Phoenix Indian Medical Center receive quality patient centered care that is consistent with the patient-centered medical home principles, and that care is timely, appropriate, cost effective, sensitive and coordinated to affect desired clinical outcomes.

The incumbent primarily works weekdays, Monday through Friday; however, may be required to work weekends and some federal Holidays and is subject to call back.

MAJOR DUTIES AND RESPONSIBILITIES

Clinical Care Duties (75%)

Implements and initiates standardized disease management and health prevention guidelines across the patient care continuum. Assess patient's needs individually and/or as a group with the care team. Collects subjective and objective health status data from multiple sources, which includes the patient, family members, team members and documented records. Analyzes health status data in terms of the unique individual and/or disease related situation. Responsible for setting up and monitoring patients enrolled in disease specific home tele-health monitoring and communicating clinical information and changes to clinical plan between patient and provider. Prioritizes the information collected based on the patient's condition and preferences, the situation, and/or anticipated needs.

Develops a plan of care with the care team according to clinical practice guidelines with the patient, family, caregiver(s) and significant other(s). Identifies resources to integrate additional clinical practice guidelines to support continuity of care. Develops and maintains patient registries based on disease, health condition and/or health prevention initiatives. Integrates patient and family education into the delivery of care. Educational activities will recognize the age, gender, race, ethnicity, diversity, and cultural norms of the individual patient and incorporate the patient and family ability to understand and participate in the plan of care.

Evaluates the outcomes of patient care and modify the plan of care based on outcomes, individually or with the team. Evaluation includes documentation of the reassessment of patient status, evidence of patient and family participation and responses, and the rationale for the revisions in the plan of care. Participates in the designing improved clinical care delivery systems and identifies opportunities and set priorities to continuously improve nursing and organizational performance through data collection and monitoring activities set by nursing, the care team and the Service Unit. Initiates therapeutic interventions and administers medications as prescribed by medical providers.

Documents nurse-patient encounters in the medical record, electronically and/or paper and use available organizational technology to enhance care, efficiency, and safety, including automated medical records and other appropriate tool. Participates in staff meetings and performance improvement studies/activities. Attends relevant in-service and education programs. Applies current nursing concepts and findings from research and/or studies to practice. Recognizes urgent/emergent patient and clinic situations and initiate emergency medical procedures and complies with external disaster, internal disaster, bomb threat and fire plans and drills. Assists with obtaining lab results, medical tests, discharge summaries and other information within the hospital and other facilities and assists with arranging appointments, procedures and discharge planning. Uses appropriate medical supplies and/or equipment for the assigned clinic, such as 12-lead EKGs, fetal monitor, pulse oximeter, etc. and collects patient specimens, including blood draw, if needed. Processes lab specimens for transport or perform tests, such as waived tests, to obtain results.

Perform other related duties as assigned.

Coordination Lead Duties (25%)

Determines which member of the nursing/health care team will provide patient care and/or services to the patient. Interventions delegated to nursing/health care team members are based on their demonstrated competency, state practice acts, available evidence-based guidelines, regulatory and governmental standards and organizational policy.

Coordinates clinic flow within the clinical care team. Coordinates and evaluates the competency and outcomes of care delegated to technical and assistive personnel.

Instructs employees in specific task and job techniques and checks on work progress and reviews completed work for accuracy and completion.

Resolves simple informal complaints and refers serious and repetitive complaints to Supervisor. Reports performance and conduct issues and progress and training needs of employees to the Supervisor.

<u>FES FACTORS</u>

Factor 1 – Knowledge Required by the Position **Level 1-6**

Knowledge of health care regulations and standards such as the Joint Commission (TJC) and Centers for Medicare and Medicaid Services (CMS) Conditions of Participation.

Knowledge of a full range of professional nursing concepts, principles, and practices and experience with clinical practice and procedures to analyze the full scope of problems associated with providing appropriate, patient centered, cost effective patient care. This clinical knowledge is applied to solve chronic health care problems related to general medical and medical specializations.

Knowledge of disease management concepts including the use of clinical practice guidelines for performance outcomes.

Knowledge of evidence based primary, secondary, and tertiary prevention protocols as it pertains to disease management.

Knowledge of a broad base of current practice in various health care delivery systems, including outpatient, emergency care, inpatient and specialty care processes and current treatment regimens and modalities.

Knowledge of institutional, community and regional health care systems to serve as an authority in matters pertaining to disease management.

Working knowledge of the operation of health care financing and Purchased and Referred Care Services related to patient referral processes, including Coordination of Benefits, Indian Health Service and the Department of Health and Human Services (DHHS) regulations, as well as Medicare/Medicaid program benefits and health care services contracts and agreements.

Ability to be self-directed and establish liaisons with inpatient/outpatient case manager/discharge planners as well as other clinical care coordinators to coordinate services throughout the continuum of care.

Ability to practice and promote customer service by promoting a customer driven environment by working effectively with patients, families, medical and internal and external health care personnel.

Ability to provide individual and family counseling, guidance and health instruction to help patients understand disease prevention and health maintenance.

Incumbent must possess and maintain a current, valid, and active state license/registration as a professional nurse.

Factor 2 – Supervisory Controls **Level 2-4**

The SCN sets overall health care objectives and defines assignments. Independently able to provide nursing care/services, education and counseling, and responds to emergencies in accordance with policy and accepted professional nursing practices without specific instruction. Work is reviewed for effectiveness of meeting clinical and administrative outcomes and

compliance with nursing and medical requirements, professional standards and Service Unit objectives and initiates.

Factor 3 – Guidelines **Level 3-3**

Guidelines consist of established objectives, policies and procedures and accepted utilization management, nursing and medical practice literature. Incumbent is expected to make recommendations regarding the efficiency and efficacy of the guidelines; however, guidelines are not applicable in every situation and independent professional judgment is required to analyze, prioritize, select, implement and determine the quality and appropriateness of the care planned for the patient(s).

Factor 4 – Complexity **Level 4-4**

Independently performs difficult and complex reviews and analysis of clinical care team data to assess and evaluate effectiveness in implementing and compliance with clinical practice guidelines. Uses considerable resourcefulness and judgment to implement modifications and select appropriate clinical practice guideline(s). Develops new and innovative processes to improve clinical care team involvement in disease management, preventative screening, and utilization of guidelines.

Factor 5 – Scope and Effect **Level 5-3**

The purpose of the work is to maximize efficiencies of health care resources planning for the needs of patients in the most appropriate setting, and to provide a seamless continuum of care. The objective is to improve the quality of care, decrease fragmentation of services and enhance patient quality of life and cost containment.

Factor 6 – Personal Contacts **Level 6-2**

The incumbent's duties and responsibilities cross all organizational lines throughout the hospital, (including medical, nursing, administrative, etc.) and also include frequent contacts with care providers in other clinical areas, other Service Units, and with contract providers and other community resources.

Primary contacts are with patients and their families and a wide range of interdisciplinary healthcare professional and administrative staff, including physicians, other medical staff providers, nurses, discharge planners, case managers, social workers, pharmacists, physical therapists, nutritionists, business office staff, PRC service staff and others. These contacts are made within Ambulatory Nursing; the Phoenix Indian Medical Center, at other Service Units and with other public and private sector providers.

Factor 7 – Purpose of Contacts **Level 7-b**

Contacts are for the purpose of motivating patients undergoing disease management services; and requires tactful approach by incumbent to carry out duties including the development and implementation of clinical practice guidelines.

Factor 8 – Physical Demands **Level 8-2**

Work requires infrequently long periods of walking, standing, and recurring of heavy lifting, such as assisting patients and carrying items like boxes of manuals and files. Work may also require driving to field clinics or other healthcare agencies.

Factor 9 – Work Environment **Level 9-2**

Work is performed in Ambulatory Nursing Services with common exposure to contagious diseases. Some degree of hazard is present in working with disturbed patients. Field clinics and health care agencies differ in physical layout.

OTHER SIGNIFICANT FACTOR

Conditions of employment – this position requires, at a minimum, Level 5 – MBI moderate risk level background investigation since the contents of the PD meets the Public Trust position criteria.

DEPARTMENT OF HEALTH & HUMAN SERVICES

Office of the
Assistant Secretary
for Administration
Office of Human
Services

HHS FLSA Classifier Checklist

Fair Labor Standards Act (FLSA) Checklist

This form is to be completed for each grade in a career ladder. Note: it is possible for different grades within a career ladder to have different FLSA designations (for example, non-exempt at lower grades, or the work is procedural in nature, and exempt at higher grades in the career ladder requiring more analytical skills to complete the tasks). A completed and signed checklist must be attached to each classified position description and statement of difference (if applicable).

Instructions: Complete the fields below and check the criteria that are applicable to the position:

A. REQUIRED POSITION INFORMATION

Position Title: Clinical Care Coordinator **Pay Plan/Series:** GS-0610-10

Position Number: See Remarks **Office/Organization:** Ambulatory Nursing

Administrative Code: Varies **Job Code:** PX3439

B. NON-EXEMPT CRITERIA

1. Salary-based non-exemption (5 CFR 551.203)

☐ Employee annual rate of basic pay is less than $23,660.

2. Non-exemption of certain employees (5 CFR 551.204)

☐ Position is non-supervisory and engaged in equipment operation, or is a protective or clerical occupation.

☐ Position involves technician work properly classified at the GS-09 level or below.

☐ Position is nonsupervisory in the Federal Wage System or other comparable wage system.

C. EXEMPTION CRITERIA

1. Executive Exemption Criteria (5 CFR 551.205)

☐ Primary duty of the position is management or supervision; work involves customarily and regularly directing the work of subordinates and has the authority to hire, remove, or evaluate performance as well as make recommendations on employment issues that are given particular weight by higher-level supervisors or managers.

2. Administrative Exemption Criteria (5 CFR 551.206)

☐ Primary duty is performance of office or non-manual work directly related to the management or general business functions. The work involves the exercise of discretion and independent judgment with respect to matters of significance.

3. Professional Exemption Criteria (5 CFR 551: 208, 209, 210)

☐ 5 CFR 551.208 Learned Professionals: Primary duty is performance of work requiring advanced knowledge in a field of science or learning including the traditional professions of law, medicine, theology, accounting, actuarial computation, engineering, architecture, teaching, pharmacy, various types of physical, chemical and biological sciences, and other similar occupations that have a recognized professional status; entrance into these professions requires a prolonged course of specialized intellectual instruction.

☐ 5 CFR 551.209 Creative Professionals: Primary duty is performance of work requiring invention, imagination, originality or talent in a recognized field of artistic or creative endeavor such as music, acting, writing, and the graphic arts. Exemption depends upon the extent of invention, imagination, originality or talent of the employee; exemption does not apply to work that can be produced by a person with general manual or intellectual ability and training.

☐ 5 CFR 551.210 Computer Employees: Primary duty consists of the application of systems analysis techniques and procedures, including consulting with users to determine hardware, software or system functions specifications; OR design development, documentation, analysis, creation, testing or modification of computer systems or programs; OR a combination of these duties. Exemption does not apply to employees engaged in the manufacture or repair of computer hardware and related equipment or to those whose work is highly reliant on computers but whose duties do not consist of the aforementioned. Salary based exemption applies to any computer professional compensated on an hourly bases at a rate of basic pay of $27.63 an hour and above.

4. Law Enforcement Activities (5 CFR 551.216)

☐ Special exemption/exclusion applies to all law enforcement agents receiving availability pay.

5. Foreign Exemption Criteria (5 CFR 551.212)

☐ Employees permanently stationed in an exempt area and spend all hours of work in a given work week in one or more exempt areas or employees not permanently stationed in an exempt area, but spend all hours of work in a given work week in one or more exempt areas. Exempt area means any foreign country, or any territory under the jurisdiction of the United States other than a State; the District of Columbia; Puerto Rico; the US Virgin Islands; American Samoa; Guam; Commonwealth of the Northern Mariana Islands; Midway Atoll; Wake Island; Johnston Island; and Palmyra (5 CFR 104).

Final Determination (check one): ☑ Non-Exempt ☐ Exempt

HR Specialist: Patricia A. Marquez -S Digitally signed by Patricia A. Marquez -S DN: c=US, o=U.S. Government, ou=HHS, ou=IHS, ou=People, 0.9.2342.19200300.100.1.1=2001337827, cn=Patricia A. Marquez -S Date: 2016.07.11 08:29:09 -07'00' **Date:** 7/11/2016

REMARKS:

Per instructions and guidance from HHS Headquarters issued on November 29, 2016 all GS-610-09/10 positions are to be deemed “Non-Exempt”. The Memorandum of Agreement has been reinstated effective November 29, 2016. The memo states “Effective immediately, you may consider the attached “Second Memorandum of Agreement LIUNA, NFFE, AFGE and IHS, dated May 21, 2010, to be in effect. All new GS-0610-09 and -10 Nurse Positions should be classified in FLSA Non-exempt status in accordance with this MOA. GS-0610-09 and -10 Nurse Positions recently classified as FLSA Exempt should be converted to Non-exempt status as soon as possible”.

Position Numbers:
00023926
00098683
00356594
00320306
00356595
00035696
00418427

Indian Health Service Human Resources Office (HRO)
Bargaining Unit Status Code Determination and Verification Recommendation

Title:	Nurse
Organizational Title:	Clinical Care Coordinator
Pay Plan/Series/Grade:	GS-0610-10
Organization:	Ambulatory Nursing Services
FLSA Status:	Non-Exempt
Administrative Code:	Varies
PD Number:	PX3439
Location:	Phoenix, Arizona
BUS Code:	1776

Notes:
A unit shall not be determined to be appropriate under this section solely on the basis of the extent to which employees in the proposed unit have organized, nor shall a unit be determined to be appropriate if it includes:

Reference: 5 USC 7112(b)(1) - (7).

1. Except as provided under Sec. 7135(a)(2) of this title, any management official or supervisor.
2. A confidential employee.
3. An employee engaged in personnel work in other than a purely clerical capacity.
4. An employee engaged in administering the provisions of this chapter.
5. Both professional employees and other employees, unless a majority of the professional employees vote for inclusion in the unit.
6. Any employee engaged in intelligence, counterintelligence, investigative or security work which directly affects national security.
7. Any employee primarily engaged in investigation or audit functions relating to the work of individuals employed by an agency whose duties directly affect the internal security of the agency, but only if the functions are undertaken to ensure that the duties are discharged honestly and with integrity.

HRO is responsible for accomplishing a BUS code review in order to process actions such as an appointment, promotion, reassignment or any other temporary or permanent action that moves an employee to a different position.

Duties that have not actually been assigned to an employee will be considered assigned duties where it can be shown that, apart from the position description, the employee has been informed the duties will be assigned, the nature of the job clearly requires the duties and the employee is not performing the duties solely because of a lack of experience. *Food Safety and Inspection Service*, 105 LRP 60863, 61 FLRA 397 (FLRA 2005); *Yuma Projects Office*, 90 FLRR 1-1455, 37 FLRA 239 (FLRA 1990).

Bargaining unit determinations are made based on duties actually performed. The PD needs to reflect the full spectrum of duties to be performed. *Pentagon Force Protection Agency*, 62 FLRA 164, 107 LRP 57744 (FLRA 2007).

An employee being provided an assignment or different position (different Position Description and Position Number), this personnel action necessitates a new review of the BUS Code.

Bargaining Unit Determination Analysis of Duties:

☑ Review duties and responsibilities assigned and determine whether or not a new position is included in the BU as described from the FLRA Certification of Representative.

Is position a professional (P) or non-professional (NP) position? ☑ P ☐ NP

Does position meet any exclusions as dictated by 5 USC 7112 (b) or is an exempt employee who administers "any provisions of the law relating to labor-management relations", 5 USC 7112 (c)? ☐ Yes ☑ No

- ☐ Management official
- ☐ Supervisor
- ☐ Confidential employee
- ☐ Employees engaged in personnel work
- ☐ Employees engaged in national security activities
- ☐ Employees involved in internal audit functions

Note: If position is a possible exclusion ensure it is researched and supported by law and/or case law; cited: N/A _____.

L/R Findings (Facts and Analysis):

Per Collective Bargaining Agreement signed September 16, 2010 the following are to be included: "All professional employees at Phoenix Indian Medical Center, Phoenix, Arizona."

After review of position description and duties, the position ☐ is or ☑ is not excluded from the bargaining unit; therefore, **BUS Code:** 1776 **applies.**

Reviewed by and date:

Patricia A. Marquez -S

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DN: c=US, o=U.S. Government, ou=HHS, ou=IHS, ou=People, 0.9.2342.19200300.100.1.1=2001337827, cn=Patricia A. Marquez -S
Date: 2016.07.11 08:31:40 -07'00'

HR Classification Specialist

7/11/2016

Date: MM/DD/YYYY

Investigate

Position Designation Record

Agency	Indian Health Service
Position Title	Clinical Care Coordinator
Series and Grade/Pay Band	GS-0610-10
Position Description Number	PX3439
Designator's Name & Title	Patricia Marquez, Classification Specialist

Potential for Compromise or Damage

Duties	Degree of Potential for Compromise or Damage
Protection of government information technology systems (supervision or control of information technology systems, authority to bypass significant technical and operational security controls for general support systems, or access to major applications – the scope of these duties exceed that of ordinary or routine computer use)	• Aids or supports the duties in this category; however, the responsibilities are so narrow in scope or carried out under such restrictive controls that the risk of violating the public's trust is practically eliminated
Protection of personal, private, controlled unclassified, or proprietary information-with the potential to damage the public's trust (includes access to or processing of personal information such as that protected by the Privacy Act (PA) of 1974, exempt from disclosure under the Freedom of Information Act (FOIA), financial data, or privileged information involving the award of contracts, contractor proprietary information, etc.)	Access and control over personal, private, proprietary, or controlled unclassified information, the unauthorized disclosure of which could negatively impact the public's trust, through serious damage/harm to: • The integrity or efficiency of the service • Individuals or business entities • Government programs or operations impacting the public's trust
Government service delivery, including customer service or public liaison duties	Duties involve customer service responsibilities and/or public liaison that could negatively impact the public's trust through moderate damage/harm to: • The integrity or efficiency of the service • Individuals or business entities • Government programs or operations impacting the public's trust

Adjustment for Program Designation and Level of Supervision

Adjustments	Label
	Agency impact

Adjustments	Label
Adjustment for Scope of Program and Correlation to Extent of Impact (see definitions)	
Adjustment for level of supervision or other controls	Periodic, ongoing review - ability to act independently a lot of the time

Total Points Designation

Label	Points
Total Initial Position Designation Points from Step 2	31
Adjusted Position Designation Points from Step 3	21

Investigation	Form Required
T2 and T2S	SF 85P

Sensitivity	Risk Level
Non-Sensitive	Moderate Risk (MB-1)

Signature: P Magley

Date: 2/8/17

Name: Patricia Magley